Clinical Guideline: Management of Type 2 Diabetes in Adults

National Health Authority (Fictional) · Document NHA-CG-01 · Version 2.1 · Issued June 2026 · Synthetic Demonstration Edition

§1 Scope

This guideline covers the management of Type 2 diabetes mellitus in adults managed in primary and secondary care. It applies to the full registered chronic-disease population and should be read together with the Cardiovascular Risk and Hypertension guidelines (NHA-CG-02, NHA-CG-03).

§2 Glycaemic targets

- A target HbA1c below 7.0% is recommended for most adults with Type 2 diabetes.
- A relaxed target of up to 8.0% may be appropriate for elderly patients, those with limited life expectancy, advanced complications, or high hypoglycaemia risk.
- HbA1c between 7.0% and 8.9% is classified as moderately controlled; 9.0% or above is classified as uncontrolled and requires treatment intensification review within 4 weeks.

§3 First-line therapy

- Metformin is first-line therapy for Type 2 diabetes unless contraindicated.
- Metformin is contraindicated when eGFR is below 30 mL/min/1.73m² and the dose should be reviewed and reduced when eGFR is below 45.
- Lifestyle intervention (nutrition, physical activity, weight management) is indicated at every stage of therapy.

§4 Treatment intensification

- If HbA1c remains at or above 8.0% on metformin monotherapy after 3 months, add a second agent selected by comorbidity.
- A GLP-1 receptor agonist is preferred for patients with BMI above 30 kg/m² or where weight reduction is a priority; it is also appropriate when HbA1c is 8.0% or above despite dual therapy.
- An SGLT2 inhibitor is preferred for patients with heart failure or chronic kidney disease stages 3a-3b, subject to eGFR of at least 20-25 depending on agent.
- Insulin should be considered when HbA1c is 8.5% or above despite two or more non-insulin agents, or at any point with symptomatic hyperglycaemia.

§5 Screening and monitoring intervals

- HbA1c: at least every 6 months when stable; every 3 months after any therapy change.
- Retinal examination: every 12 months for all patients with diabetes.
- Foot examination: every 12 months; high-risk feet every 3-6 months.

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- Renal profile (eGFR and albumin-creatinine ratio): every 12 months, or every 6 months in CKD stage 3a or worse.
- Lipid profile: every 12 months, aligned to the Cardiovascular Risk guideline.

A patient whose interval is exceeded is recorded as having an open care gap and enters the recall workflow defined in the Population Screening and Recall Policy (NHA-PP-01 §4).

§6 Renal considerations

- Review all medication during acute kidney injury; withhold metformin and SGLT2 inhibitors while eGFR is acutely below 30.
- Refer to nephrology at CKD stage 4 (eGFR below 30) or with rapidly declining function.

§7 Human oversight of AI-assisted recommendations

Any medication recommendation produced with the assistance of an analytical model or AI agent is advisory only. A licensed clinician must review and approve the recommendation before any prescription is issued. AI-generated drafts must be recorded with their evidence citations in the audit log.

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